INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Fulvestrant · injections every 4 weeks

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Regimen source
NCCP 00361
Version 4

Loading visits, then every 28 days

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	10
04	Sources and further information	15

My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

The aim is to control advanced hormone-sensitive breast cancer and its symptoms, with response and tolerability reviewed before continuing.

HORMONE THERAPY · OESTROGEN-RECEPTOR DEGRADER

Fulvestrant

Binds the oestrogen receptor, blocks its signalling and helps the cell remove the receptor

Oestrogen receptors can drive cancer-cell growth → Fulvestrant binds and blocks the receptor → The number of receptors falls, reducing growth signals

Fulvestrant is a selective oestrogen-receptor degrader. It blocks receptor activity and lowers the amount of receptor available in cancer cells. The slow-release injection keeps the medicine in the body between visits; the extra first-month dose helps establish treatment exposure.

Report severe injection pain, numbness, weakness or pain travelling down the leg, unusual bleeding or bruising, jaundice and symptoms that interfere with daily life.

The treatment being discussed

Who this information covers

This NCCP monotherapy pathway covers postmenopausal women with ER-positive locally advanced or metastatic breast cancer that has relapsed or progressed on or after adjuvant anti-oestrogen treatment. Fulvestrant prescribed with capivasertib or a CDK4/6 medicine follows a separate combination regimen.

NCCP 00361 v4 · official regimen

Medicine and schedule

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days. Each usual 500 mg dose is given as two 5 mL injections, one in each buttock, slowly over one to two minutes per injection. This is not an intravenous infusion; no port or pump is needed for fulvestrant itself.

NCCP 00361 v4 · official regimen

Review and duration

Each usual 500 mg dose is given as two 5 mL injections, one in each buttock, slowly over one to two minutes per injection. This is not an intravenous infusion; no port or pump is needed for fulvestrant itself. Symptoms can occur at any time; this is not a chemotherapy nadir or recovery curve.

NCCP 00361 v4 · official regimen

Individual risks and support

Discuss the treatment aim, injection discomfort and possible nerve injury, bleeding or bruising, liver effects, flushes, joint symptoms and fatigue. Liver impairment or an existing modified dose requires an agreed prescriber and pharmacy plan. Where relevant, use effective contraception during treatment and for two years after the final dose under the EMA product information. Pregnancy and breastfeeding are excluded. Tell staff about blood thinners; do not stop them yourself.

NCCP 00361 v4 · official regimen

Why fulvestrant is offered

Fulvestrant can help control advanced hormone-sensitive breast cancer. Studies support the usual treatment schedule, while the choice of treatment depends on previous endocrine therapy and the individual cancer.

A study comparing fulvestrant doses does not show that monotherapy is better than modern combination treatment. A separate study in people who had not received endocrine therapy found longer control of progression but no proven overall-survival advantage over anastrozole. These findings do not predict an individual outcome.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Tell the team about unusual bleeding or extensive bruising, injection-site pain with numbness or pain down the leg, jaundice or worsening symptoms. Severe chest pain, breathlessness or stroke symptoms require emergency help.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Read the row for your treatment setting. Columns show order, not equal lengths of time. Agree the due window, booking team, result reviewer and whom to contact if a check is missing.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00361 v4 · official regimen · EMA Faslodex product information · Pan-Asian adapted ESMO early breast cancer guidelines · Pan-Asian adapted ESMO metastatic breast cancer guidelines · NICE NG 101: early and locally advanced breast cancer · ESO-ESMO ABC 5 consensus: tumour-marker interpretation

THE WHOLE TREATMENT JOURNEY

Fulvestrant · injections every 4 weeks: treatment and checks

Your treatment pattern

Loading visits, then every 28 days

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Postmenopausal ER-positive locally advanced or metastatic breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Treatment plan NCCP 00361	Confirm indication and prescription	Start the agreed daily / injection schedule	Continue unless prescriber changes the plan	Review duration, benefit and tolerability	Agree continuation, switch or completion
Bloods / symptom review NCCP 00361	Blood count, kidney and liver profile; review medicines and bleeding risk	Confirm Day 15 loading visit; assess injection tolerability	Liver profile when indicated; review bleeding and nerve symptoms	Clinical and imaging review of benefit and tolerability	Follow unresolved symptoms or test abnormalities
electrocardiogram (ECG) / echo NCCP 00361	No routine ECG / echo for this medicine alone	Investigate new cardiac symptoms	Further cardiac tests by risk / symptoms	Companion-drug cardiac rules remain separate	Follow abnormalities when indicated

Care / check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Bone density / dual-energy X-ray absorptiometry (bone density) (DXA) NCCP 00361	Assess personal fracture / osteoporosis risk	DXA only when indicated	No cycle-based DXA appointments	Repeat only according to the bone-risk plan	Continue bone care according to risk
Advanced-disease response NCCP 00361	Document disease sites / baseline assessment	New symptoms can bring assessment forward	Generally every 2–4 months; individualise	Adapt interval and imaging to disease / benefit	Markers supplement, never replace, assessment
Oestrogen blood tests NCCP 00361	Tell the team that fulvestrant is planned	Some tests can read falsely high	Confirm a suitable test with the laboratory if needed	Do not change treatment on a misleading result	Tell later clinicians about prior fulvestrant

What each check is for

1 Confirm the plan

Before starting or changing treatment

This NCCP monotherapy pathway covers postmenopausal women with ER-positive locally advanced or metastatic breast cancer that has relapsed or progressed on or after adjuvant anti-oestrogen treatment. Fulvestrant prescribed with capivasertib or a CDK4/6 medicine follows a separate combination regimen.

Ask / act: Confirm treatment setting, medicines, menopause and the planned review.

2 Checks and symptoms

At the agreed review points

Before treatment, check blood count, kidney and liver function, the menopausal and treatment history, pregnancy exclusion and bleeding risk. Before injections, tell staff about anticoagulants, bruising, bleeding or previous nerve symptoms. Repeat liver tests when indicated; scans follow the agreed cancer-control plan. Tell anyone ordering an oestrogen blood test that you receive fulvestrant because some tests can give a misleadingly high result.

Ask / act: Agree who books tests, reviews results and responds to new symptoms.

3

Review ongoing benefit

At setting-specific decision points

The aim is to control advanced hormone-sensitive breast cancer and its symptoms, with response and tolerability reviewed before continuing.

Ask / **act:** Review duration, alternatives and patient priorities; do not stop or extend treatment automatically.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks throughout treatment

Your team arranges tests and explains the results. A planned check is not confirmation that it has been completed.

Treatment-specific checks

Before treatment, check blood count, kidney and liver function, the menopausal and treatment history, pregnancy exclusion and bleeding risk. Before injections, tell staff about anticoagulants, bruising, bleeding or previous nerve symptoms. Repeat liver tests when indicated; scans follow the agreed cancer-control plan. Tell anyone ordering an oestrogen blood test that you receive fulvestrant because some tests can give a misleadingly high result.

NCCP 00361 v4 · official regimen

Heart checks

ECG or other heart tests depend on symptoms and personal risk; this medicine alone does not require a routine HER2-style echocardiogram schedule.

NCCP 00361 v4 · official regimen

Bone health

Assess personal fracture and osteoporosis risk. Fulvestrant does not carry an automatic aromatase inhibitor (AI) DXA timetable; agree testing and repeat intervals when indicated.

NCCP 00361 v4 · official regimen · NICE NG 101: early and locally advanced breast cancer

Your setting changes the pathway

Choose the pathway that your team has confirmed; these are not all extra treatments you must have.

Advanced-disease scans

For advanced breast cancer, response assessment is generally every two to four months, adapted to disease pace, sites, symptoms and treatment. Indolent disease may be reviewed less often; new symptoms can prompt earlier scans. The team chooses the imaging method; this is not a universal three-month computed tomography (CT) chest/abdomen/pelvis order.

Tumour markers

Tumour markers may supplement imaging and clinical assessment in advanced disease if they are informative, especially if elevated at baseline. They do not replace scans, and a marker change alone should not determine treatment failure. There is no universal fixed marker timetable.

Menopause, pregnancy and contraception

Where relevant, effective contraception is needed during treatment and for two years after the final dose under the EMA product information. Pregnancy and breastfeeding are excluded.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Your endocrine treatment and review milestones

The first month has an extra injection visit. The later interval is every 28 days, not automatically the same date each calendar month.

Treatment or check	Timing in this view
Injection visits	**Day 1** Usual dose 500 mg
Extra loading visit	**Day 15** Usual dose 500 mg
Every 28 days	**Day 29 → Day 57 onward** Day 29, Day 57, then every 28 days
Safety / response review	**Before starting → Day 1 → Day 15 → Day 29 → Day 57 onward** Bloods at baseline; later tests and scans follow the individual plan

Each usual 500 mg dose is given as two 5 mL injections, one in each buttock, slowly over one to two minutes per injection. This is not an intravenous infusion; no port or pump is needed for fulvestrant itself.

Symptoms can occur at any time; this is not a chemotherapy nadir or recovery curve.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Practical support

Make injection visits manageable

Discuss injection discomfort, flushes, joint or muscle symptoms, fatigue and nausea. Ask early for symptom help or practical support attending appointments.

Support medicines

Fulvestrant alone does not require routine steroid premedication, injections to raise white cells or a standard anti-sickness course. A combination medicine can add its own supportive-care needs.

Bleeding and nerve symptoms

Tell staff about blood thinners, unusual bruising or bleeding before an injection. Severe buttock pain, new numbness, weakness or pain down the leg needs prompt assessment.

Tests and appointments

Confirm the extra loading visit and later appointments, who books required blood tests and scans, and who reviews the results. Tell the laboratory team about fulvestrant if an oestrogen blood test is ordered.

Taking tablets / attending injections

Questions to ask

When are the injections given?

The usual schedule is Day 1, Day 15 and Day 29, then every 28 days. The first month includes an extra loading visit. Follow your clinic appointments; contact the team if a visit is missed.

When are bloods, ECGs or echoes needed?

Before treatment, check blood count, kidney and liver function, the menopausal and treatment history, pregnancy exclusion and bleeding risk. Before injections, tell staff about anticoagulants, bruising, bleeding or previous nerve symptoms. Repeat liver tests when indicated; scans follow the agreed cancer-control plan. Tell anyone ordering an oestrogen blood test that you receive fulvestrant because some tests can give a misleadingly high result. There is no routine ECG or echo schedule for fulvestrant alone; a companion medicine may need additional checks.

How long will treatment continue?

Continue while the treatment is beneficial and tolerated, with planned response reviews. There is no fixed adjuvant five-year course on this card.

Do I need a port or home pump?

No port or home infusion pump is needed for this medicine itself. Any separately prescribed medicine has its own access requirements.

Is this the same plan as capivasertib with fulvestrant?

No. The injections are part of both plans, but capivasertib adds its own tablet schedule, selection requirements, blood-sugar checks and side-effect care. Use the full combination plan if both are prescribed.

Why might an oestrogen test need checking?

Fulvestrant can interfere with some laboratory tests and make the result look higher than it is. Tell the team about treatment so they can check the test method before making a treatment decision.

Explain the plan back: What is your treatment schedule, which checks apply to your setting, and who books and reviews them?

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

Clinician supportive-care discussion draft. This is not an issued prescription.

Minimal emetogenic risk

No routine prophylactic antiemetic, corticosteroid premedication or granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) order is populated for this endocrine monotherapy. Review persistent nausea or other symptoms and prescribe individually if needed.

Medicine-specific support

- Discuss injection discomfort, flushes, joint or muscle symptoms, fatigue and nausea. Ask early for symptom help or practical support attending appointments.
- Fulvestrant alone does not require routine steroid premedication, injections to raise white cells or a standard anti-sickness course. A combination medicine can add its own supportive-care needs.
- Tell staff about blood thinners, unusual bruising or bleeding before an injection. Severe buttock pain, new numbness, weakness or pain down the leg needs prompt assessment.
- Confirm the extra loading visit and later appointments, who books required blood tests and scans, and who reviews the results. Tell the laboratory team about fulvestrant if an oestrogen blood test is ordered.
- Discuss the treatment aim, injection discomfort and possible nerve injury, bleeding or bruising, liver effects, flushes, joint symptoms and fatigue. Liver impairment or an existing modified dose requires an agreed prescriber and pharmacy plan. Where relevant, use effective contraception during treatment and for two years after the final dose under the EMA product information. Pregnancy and breastfeeding are excluded. Tell staff about blood thinners; do not stop them yourself.
-

Individual plan

Prescriber / date: _____ Pharmacy review: _____

Verify allergies, interactions, relevant organ function, treatment setting and the exact medicine before prescribing.

NCCP 00361 v4 · Monitoring and treatment map

Fulvestrant · injections every 4 weeks

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

Hormone therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Injection area

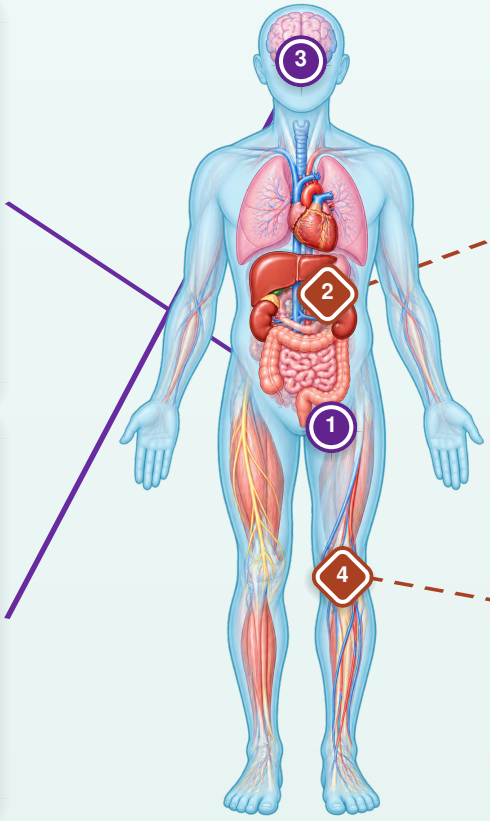
Fulvestrant

Report severe gluteal pain, numbness or pain travelling down the leg.

2 Liver

Fulvestrant

Report jaundice, dark urine or marked new illness; keep indicated liver tests.



3 Hot flushes

Fulvestrant

Tell the team if flushes or sweats disturb sleep and daily life.

4 Joints / muscles

Fulvestrant

Report persistent pain or symptoms limiting activity.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Tell the team about unusual bleeding or extensive bruising, injection-site pain with numbness or pain down the leg, jaundice or worsening symptoms. Severe chest pain, breathlessness or stroke symptoms require emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00361 v4 · official regimen**
   https://healthservice.hse.ie/documents/6816/361_Fulvestrant_Therapy.pdf
2. **EMA Faslodex product information**
   https://www.ema.europa.eu/en/documents/product-information/faslodex-epar-product-information_en.pdf
3. **Pan-Asian adapted ESMO early breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC11145753/
4. **Pan-Asian adapted ESMO metastatic breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC10186487/
5. **NICE NG 101: early and locally advanced breast cancer**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
6. **ESO-ESMO ABC 5 consensus: tumour-marker interpretation**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC7510449/
7. **CONFIRM: final overall survival, fulvestrant 500 mg versus 250 mg**
   https://pubmed.ncbi.nlm.nih.gov/24317176/
8. **FALCON: fulvestrant versus anastrozole in endocrine-naive advanced breast cancer**
   https://pubmed.ncbi.nlm.nih.gov/27908454/
9. **FALCON: prespecified final overall survival**
   https://pubmed.ncbi.nlm.nih.gov/39772884/